Cardivex Post-Marketing Safety Surveillance Report, Year 1

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Scope

This periodic safety report summarizes spontaneous and registry-based adverse event reports for Cardivex during its first year of fictional marketing. Estimated exposure was 58,300 patient-years across 41,900 treated patients.

Key findings

Hyperkalemia was reported in 2.1% of registry patients, higher than the 1.8% observed in the CARDIO-ALPHA trial; most cases occurred in patients with eGFR below 45 mL/min/1.73 m2 or concomitant potassium-sparing diuretics. Angioedema incidence was 0.4% (176 cases), of which 12 required emergency airway management. Dry cough leading to discontinuation was reported in 2.6% of patients.

Signal assessment

No new safety signal was identified. The angioedema rate is consistent with the trial estimate. The hyperkalemia signal is considered expected and is linked to renal impairment.

Risk minimization

The label was updated to recommend serum potassium and creatinine monitoring 2 to 4 weeks after initiation in patients with chronic kidney disease stage 3 or higher.

Summary table

Adverse event	Reports	Incidence
Hyperkalemia	880	2.1%
Cough leading to discontinuation	1,089	2.6%
Angioedema	176	0.4%
Acute kidney injury	67	0.16%